tween two extremes with ADHD—either we overdiagnose or we underdiagnose." Adds Dr Sachin Bagla, psychiatrist at Fortis Bengaluru, "As individuals grow older and life becomes more complex, the demands on attention, organisation and emotional regulation increase. That is when longstanding difficulties become more visible, showing up as procrastination, forgetfulness, last-minute work patterns, reliance on caffeine or screens, and emotional dysregulation."

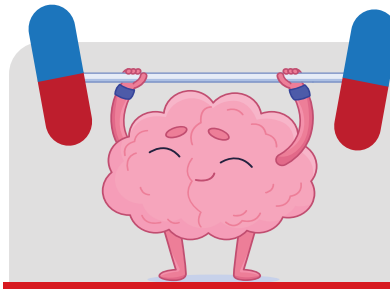
DISTRACTION GETS ATTENTION

One reason why ADHD is appearing in conversations today is social media. Indian creators like Sana Khan (@adhd.india), Riya Kothari (@adhd_coach_india) and other mental health influencers are using formats like short-form videos, reels and explainers to make the condition more visible, while breaking down everyday behaviours—procrastination, forgetfulness, impulsivity—for viewers to connect patterns in their own lives to possible underlying issues.

While these videos are not diagnostic tools, they have become an entry point. Among the youngest voices in this space is 18-year-old Aaria Verma, who began sharing her experiences on YouTube after her own diagnosis. "A lot of symptoms get missed because you don't realise that a part of your behaviour could be linked to a condition," she says. "When you talk about it, people start recognising patterns in themselves."

Clinicians, though, caution against equating accessibility with accuracy. "A lot of people are coming in convinced they have ADHD after watching something online, but that is not always the case," says Chaddha. Adds Bagla: "Not all attention problems in adults are ADHD. Anxiety, depression and lifestyle factors like excessive screen use can impair focus in similar ways. That is why a proper evaluation—looking at childhood patterns, screen habits, addictions and underlying mental health issues—is essential to make a diagnosis."

Part of the problem also is that ADHD is not diagnosed the way most people imagine. There is no single blood test or scan to confirm it. "You cannot diagnose ADHD from a reel, a self-test," says Dr Parikh. "You must look at the person's life over time—how they function under structure and without it." A person who suspects ADHD should see a psychiatrist or a clinical psychologist familiar with neurodevelopmental conditions. Their assessment will involve history-taking, symptom-mapping and use of diagnostic criteria such as those described in the Diagnostic and Statistical Manual of Mental Disorders. When possible, inputs from parents, caregivers, teachers or partners can help.



TREATMENT OF ADULT ADHD

COGNITIVE BEHAVIOURAL THERAPY *(CBT)*
Helps fix issues with deadlines, procrastination and forgetfulness by fragmenting large tasks and externalising memory through reminders

MEDICATION
Improves attention and impulse control. Stimulants such as methylphenidate and non-stimulants such as atomoxetine are commonly prescribed

STRUCTURAL AND LIFESTYLE CHANGES
Consistent routines, adequate sleep and exercise help, as does sensitisation among peers

Self-diagnosis, on the other hand, can be misleading. Many traits associated with ADHD—difficulty concentrating, impulsivity, restlessness—are experiences most people have at some point. But undergoing a symptom occasionally is not the same as having a clinical condition. The difference lies in persistence, severity and impact on daily functioning. "The kind of lifestyle we lead today also affects attention—constant multitasking, rushing, switching between tasks," says Parikh. "Making a mistake because I am distracted in the moment does not mean I have ADHD, it may just mean I did not have the right attention at the time." That balance—between recognition and over-attribution—is central to how ADHD is understood today. "ADHD needs to be identified early when the clinical condition is evident, and intervention should begin at the right time. But we must also distinguish between clinical ADHD and lifestyle-linked attention issues," says Parikh.

TREATING ADHD

Doctors recommend a mix of behavioural therapy, environmental redesign and, where clinically indicated, medication. Cognitive behavioural therapy (CBT), in particular, has emerged as a central pillar of treatment. "The aim is not to 'fix' a person, but to create systems that help them function, learn and work more effectively," says Dr Rajender Singla, senior consultant, neurology, at Sarvodaya Hospital, Faridabad. CBT for adults includes executive function strategies: learning how to split large tasks into smaller steps, externalising memory through planners and reminders, creating fixed routines, reducing competing distractions, building pause techniques before impulsive reactions, rewards upon completed tasks, accountability check-ins and short breaks.

Medication, meanwhile, has evolved from 'sedating' or 'dependency-creating' psychiatric drugs to stimulants (like methylphenidate) and non-stimulants (like atomoxetine) to increase dopamine and norepinephrine levels. These hormones and neurotransmitters improve focus and impulse control.

Sanyal is already feeling the difference post diagnosis and treatment. For her, therapy is "learning how to work with my brain instead of against it". Her condition has not disappeared, but she has learnt to tame the beast. ■